جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : مني محمد احمد ++
المحافظة : القاهره **الحي / المدينة :** طره **العنوان :** ++++
رقم البطاقة : 28006180102926 **رقم التليفون :** 01220033729 **تاريخ تسجيل الإستمارة :** 19-10-2026
العمر(تاريخ الميلاد) : 18-06-1980 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: مني محمد احمد ++
National ID: 28006180102926

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحقيق الشخصية

منى
محمد احمد عثمان
٢٣ شارع الوفاء والامل-طره الاسمنت
طره - القاهره

٢٦ ٢٩ ٠١ ٠ ١٨ ٠٦ ٠ ٢٨

JW2587971

National ID · Front
[[DOCTYPE:ID_FRONT]]

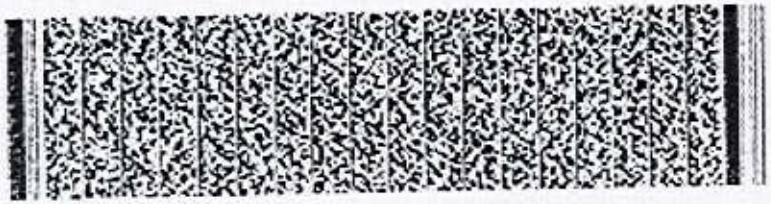
٢٨٠٠٦١٨٠١٠٢٩٢٦ ٢٠٢٣/٠٢

رية منزل

أنثى مسلمة متزوجة

شحاته محمود هيبه عبدالله

البطاقة سارية حتى ٢٠٣٠/٠٢/١٧



National ID - Back
[[DOCTYPE:ID_BACK]]

Name: منى محمد احمد عثمان	
MR. : 4848	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient was previously diagnosed with premenopausal metastatic breast cancer involving lung and lymph nodes, with recent development of bone metastases (IDC2, ER-ve, PR 3/8, HER2+, Ki-67 35%). The oncologic history includes right-sided breast cancer diagnosed in 2023, staged as cT4b N2, for which the patient received six cycles of taxotere and carboplatin without anti-HER2 therapy. Post-operative radiotherapy was administered in September 2023, followed by progression with right internal mammary and left axillary lymph node involvement and pulmonary nodules. From October 2023 to January 2024 the patient was treated with capecitabine (Xeloda) and trastuzumab, then lost to follow-up until September 2024 when disease progression prompted initiation of gemcitabine (Gemzar) plus pertuzumab (Tykerb), administered for seven cycles until February 2025. After another loss to follow-up, the patient received weekly paclitaxel (Taxol) for 22 weeks, concluding in February 2025. Imaging on 30 March 2026 demonstrated progressive disease of the left breast and axillary nodes, sternal and right first-rib soft-tissue components, and new lytic lesions of the D1 and D2 vertebral bodies, prompting a multidisciplinary team (MDT) review. The patient has completed six cycles of taxotere/carboplatin, seven cycles of gemcitabine/pertuzumab, and 22 weeks of weekly paclitaxel.

On examination, the patient exhibited right chest-wall skin nodules and right cervical lymphadenopathy. The MDT in May 2026 recommended rechallenge with capecitabine (Xeloda), trastuzumab, and zoledronic acid (Zometa) on a 28-day schedule, targeting the right first-rib soft-tissue component. Dental evaluation was pending approval of trastuzumab. The prescribed capecitabine regimen was 3 × 2 × 14 (WT 65 HT162 SA 1.7). The patient has completed three cycles of capecitabine as described.

The patient presented for a follow-up on 22 August 2026, receiving capecitabine (C2) at the same 3 × 2 × 14 dosing schedule. Laboratory results were acceptable for proceeding with the planned dental consultation prior to zoledronic acid administration. The current plan is to continue the capecitabine regimen pending dental clearance.

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

Patient's Name:	**mona mohamed ahmed**
Date:	**2026-03-30**
Referred by:	Prof.Dr. **ct cap iv**
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

A known case of managed right breast cancer

Compared to last study dated 13-4-2025

I- CHEST:

- Right MRM showed clear operative bed with no enhanced focal lesions or fluid collection.
- The left breast showed UOQ soft tissue mass lesion shows progressive coure a regard the size measures about 7 x 2.6 compared to 6.2x2.35 cm causing skin retraction and associated with enlarged ipsilateral axillary lymph nodes, the largest measures about 2x1.7 cm **for further sono-mammography**
- Bilateral mosaic parenchymal pattern with air trapping denoting small air way disease.
- Left apical and right lung lobes thick atelectatic bands are noted
- No pathological mediastinal lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.
- Enlarged right copula of diaphragm.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous fatty parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.

CT Scan
[[DOCTYPE:CT_SCAN]]

جوستاف روسي

Patient's Name:	**mona mohamed ahmed**
Date:	**2026-03-30**
Referred by:	Prof.Dr. **ct cap iv**
Hospital:	**Goustave Roussey International Egypt**

- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Right caval lymph node measures about 16x11.7mm with multiple few aortic lymph nodes.
- No ascites.
- **The examined skeleton**
  - Progressive course as regard The previously noted sternum and right 1st rib soft tissue mass component measures about 6 x3 cm compared to 5x4.5 cm at sternum and 5.5x4.5 comapared to 5.5 x4 cm at 1st rib.
  - Lytic lesions with selrotic margins seen at cervical vertebral and D1,D2 vertebral bodies.
  - **For further bone scan**

Opinion:

- **STILL SEEN Bilateral mosaic parenchymal patterns with bilateral thick atelectatic bands.**
- **Few abdominal lymph nodes enlargement**
- **Progressive course as regard the Left breast mass lesions with enlarged axillary lymph nodes Sternum and right 1st rib lytic lesions with soft tissue component likely metastasis**

Dr.Norhan Ramzy,MD
Dr Abubakr Hejazi MSc FEVR

Dr.Nourhan Ramzy

Dr.Nourhan Ramzy

CT Scan
[[DOCTYPE:CT_SCAN]]

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنك المصري لصحة مصر

05/11/2026

MDT FORM

Name:	Monna Mohamed El-Sayed Othman	MRN:	4848
Age:	46 Y	ID:	28006180102926
Menopausal Sta	prem	Phone:	01104394198
NRS SCORE:	3	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	

Clinical Data:

BX:

ER: PR: HER2: KI67:

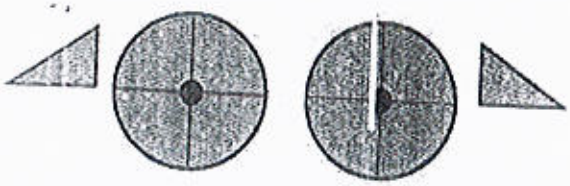
Sonomammography:

Mets Workup:

History:

premenopausal met breast cancer to lung lymph nodes recently newly developped bone mets idc2 er-ve pr3/8 her23+ ki 35% .h story 2023 rt breast cancer ct4b n2 mo rec taxotere carboplatin x6 cycles no anti her2 given rt mrm 9/2023 ypt4 ypn3 posto cts newly dev rt internal mammary ln left axill ln pd of pulmonary nodules rec xeloda and herceptin from 10/2023 to 1/2024 then lost fu till 9/2024 pd so st gemzar tykerb from 9/2024 to 2/2025 rec 7 cycles then lost fu till8/2025 dp st taxol rec 22 weekslast 2/2025 cts on 30/3/2026 progressive course of left breast and axillary ln pd of sternal and rt 1st rib soft tissue component and newly developped lytic lesions d1,d2 vertebral bodies for mdt bs srn requested

R L



TNM

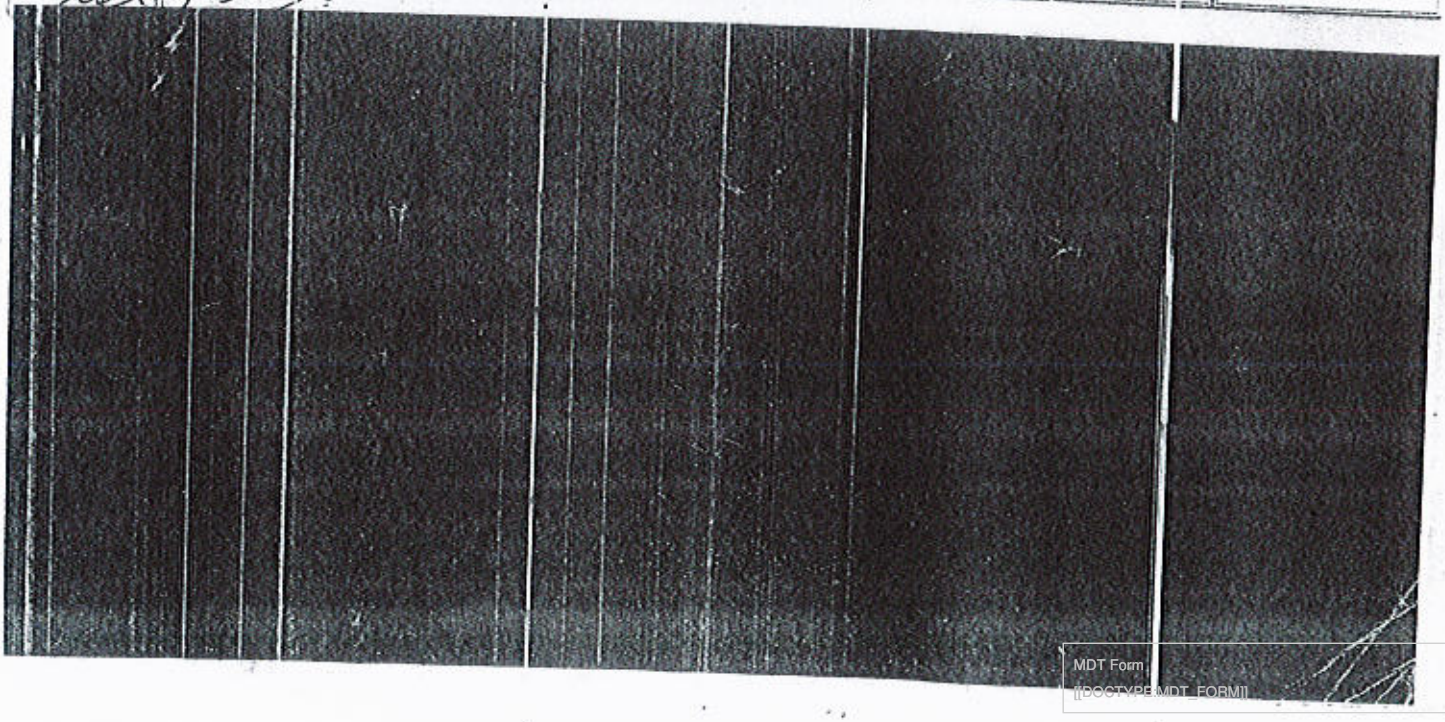
Recommendations:

MDT Outcome & Plan

Hercepitan RECHALLENGE WITH XELODA
please check peripheral neuropathy ZOMETA EVERY 28 DAYS
CHECK FOR PALIATIVE RTH ON 1ST RIB FOR SOFT TISSUE COMPONENT

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ نرمين محمد في		د/ ماهر مختار			



MDT Form
[[POSTYPE:MDT_FORM]]

MDT form

3- Medical number: 83126563
Old Med : 596/2023

A-Patient ID
1- Name :

Sex: Female

Patient Age : 43 Y

2-ID number: 28006180102926

REASON OF MDT

Family history

Phone 01104394198

BMI

Date of MDT 2025-05-13

marital status

NRS Score

GROUP

Co morbidity

Menopausal status

Initial STAGE

Clinical data

Breast Imaging :

Mets Workup :
CTSCAN-->LT TINY NODULES,,DVG SCLEROTIC LESION

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	2/8	3/8	3		35

T	N	M
4B	3	1

Sonomammography

PATIENT HISTORY
PT RECEIVED 6 CYCLES TAXOTERE/CARBO ,,NO APROVAL FOR HP,,SUBJECTIVE
IMPROVMENT BUT
STILL BREAST LESION
RT MRM:
free margins
free nipple . areola
LN 13/15
partial pathological response
YPT4 YPN3
NEW CT CAP :PROGRESSION OF RT INT MAMM LNS, ENLARGED LEFT AXILLARY LNS,
RT SUPRACLAV LNS NEWLY DEVELOPED, LUNG NODULES
pt received 4c xeloda HERCEPTIN
Gemzar & Tykerb 7 CYCLES

MDT OUTCOME & PLAN

- For REBIOPSY
START AC UNTIL BX RESULTS

SIGNATURE

RADIOLOGY

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name: Mona Mohamed Ahmed Othman	Date : 13/04/2025
Exam : C.T CHEST WITH CONTRAST	Age : 45 Year(s)
Medical ID: 596/2023	Pat. ID: 28132

Post-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

*****As Compared to the previous study dated last study 08-12-2024, current study shows:***

Chest:

- Right mastectomy showing no gross operative bed masses or collections.
- **Mixed course** regarding the size and increase in number of the enlarged left axillary lymph nodes, the largest measures about 5.1 9X3.5 cm compared to 4.5x3.6 cm and seen inseparable from the overlying skin.
- **progressive course** regarding the size of right internal mammary LNs, the largest measures 4X 3.4 cm compare to 1 cm currently infiltrating the sternum with extraosseous soft tissue component.
- **Progression in size of the** Left upper lung lobe apico-posterior nodules the largest measures 3.7X4.3 cm compared 2.7x2.2 cm previously.
- **Progression of enlarged** right supraclavicular the largest measures about 4.6x 2 cm with currently seen infraclavicular nodes ? nodular density
- **Still noted** mildly enlarged right axillary lymph nodes,.
- **Newly developed** the right middle and lower subsegmental atelectatic bands
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.

Abdomen and pelvis:

- The liver is enlarged with homogenous CT texture. No definite focal lesions or biliary radicles dilatation
- Normal size & parenchymal texture of the spleen with no focal lesions seen, No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal non contrast appearance of the pancreas and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

The scanned skeleton:

- Stable D6 vertebral body sclerotic area is seen with heterogenous osseous texture of the examined bones.

OPINION:

- **Status post right mastectomy showing**
  - **Mixed course regarding left axillary lymph nodes**

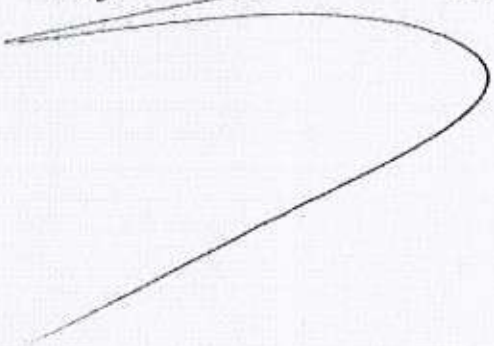
CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name: Mona Mohamed Ahmed Othman	Date : 13/04/2025
Exam : C.T CHEST WITH CONTRAST	Age : 45 Year(s)
Medical ID: 596/2023	Pat. ID: 28132

- Progression of pulmonary nodules and right internal mammary lymph nodes.
- Progression of right supraclavicular lymph nodes .

Dr. Fatima Ahmed

Prof. Menna Tallah Hanafy, M.D.



CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name	: الأستاذه / منى محمد احمد عثمان	Request No.	: 13007874
Patient No.	: 28006180102926	Request Date	: 01/08/2024
Path No.	: 2696/24	Verification Date	: 07/08/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 44 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast malignant looking solid mass lesion.
Nature of Specimen: Tru-cut biopsy, collected at 1/8/2024.

Gross Examination:
- Multiple grayish white tissue cores up to 1.8 cm, totally submitted.

Microscopic Examination:
- Histological type: Focally necrotized invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- Ductal carcinoma in situ: Negative.
- Perineural invasion: Negative.
- Angioinvasion: Negative.
- Microcalcifications: Negative.

Diagnosis: (Revised)
Left breast malignant looking solid mass lesion, Tru-cut biopsy:
Focally necrotized invasive duct carcinoma, Grade II.
For further IHC (ER, PR, Her2, Ki67).

Signature
Dr. Walaa Ghonaim.
Dr. Toka Medhat.

Verified by :
Dr.Toka Medhat

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / منى محمد احمد عثمان	Request No.	: 13007874
Patient No.	: 28006180102926	Request Date	: 01/08/2024
Path No.	: 2696/24	Verification Date	: 24/08/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 44 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast malignant looking solid mass lesion.
Nature of Specimen: Tru-cut biopsy, collected at 1/8/2024.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & KI67 clone (30-9).

Results:
* IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction in the tumor cells (score 0/8).
- PR: Weak nuclear reaction in 1% of tumor cells (score 2/8).
- Her2: Strong complete membrane reaction in >10% of tumor cells (score 3+).
- Ki67: 25% nuclear reaction of tumor cells.

Conclusion:
Left breast malignant looking solid mass lesion, Tru-cut biopsy:
Focally necrotized invasive duct carcinoma, Grade II (revised).
IPT:
- ER: Negative (score 0/8).
- PR: Negative (score 2/8).
- Her2: Positive (score 3+).
- Ki67: Positive in 25% of tumor cells.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1,

Verified by :
Dr.Toka Medhat

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Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذه / منى محمد احمد عثمان	Request No.	13007874
Patient No.	28006180102926	Request Date	01/08/2024
Path No.	2696/24	Verification Date	24/08/2024
Sex	Female	Financial Category	ورام دار السلام - ميادره
Age	44 Y	Department	Out Patient
Referreing Doc.			
		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

>1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Walaa Ghonaim.
Dr. Toka Medhat.

Verified by :
Dr.Toka Medhat

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
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Immunohistochemistry
[[DOCTYPE:IHC]]

[[DOCTYPE:MDT_FORM]]
MDT Form

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

10/14/2026

MDT FORM

Name:	Monna Mohamed El-Sayed Othman	**MRN:**	4848
Age:	46 Y	**ID:**	28006180102926
Menopausal Sta		**Phone:***	01125117064
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
Gender			
CO Morbidity:		**Reason Of MDT:**	DP metastatic
Surgical History:*	Rt MRM in 2023	**ECOG Performance Status**	○ 0 ○ 1 ◉ 2 ○ 3 ○ 4

Clinical Data:

BX:

ER: 0/8 **PR:** 3/8 **HER2:** +3 **KI67:**

History:

premenopausal met breast cancer to lung lymph nodes recently newly developped bone mets idc2 er-ve pr3/8 her23+ ki 35% .
2023 rt breast cancer ct4b n2 mo rec taxotere carboplatin x6 cycles no anti her2 given rt mrm 9/2023 ypt4 ypn3 postop cts newly dev rt internal mammary ln left axill ln pd of pulmonary nodules rec xeloda and herceptin from 10/2023 to 1/2024 lost fu till 9/2024 pd so st gemzar tykerb from 9/2024 to 2/2025 rec 7 cycles then lost fu till8/2025 dp st taxol rec 22 weeks last 2/2025 cts on 30/3/2026 progressive course of left breast and axillary ln pd of sternal and rt 1st rib soft tissue component and newly developped lytic leslons d1,d2 vertebral-Received Xeloda and was planned for Herceptin (Not received not approved) now she has skin nodules progression +Ulcers and poor performance.

Sonomammography:

Biomarker/Molecular

Mets Workup:

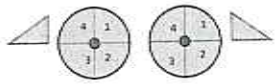
Recommendations:

For New CT CAP assessment R
Possibility of new Biopsy shift to IV Chemo single agent : Gemzar ?
Palliative radiation to skin nodules -- RIb lesion after Pt assessment at OPC

MDT Outcome & Plan

Review CT CAP new Biopsy from skin nodules
Porta cath and shift to Gemzar single agent.
Palliative radiation to skin nodules -- RIb lesion after Pt assessment at

R L



TNM

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د ندي حسن	د.امنيه عيسوي		

14/10/2026